

Jawaharlal Nehru Medical College Hospital

JNMC Campus, Sawangi (Meghe), Wardha - 442004, Maharashtra, India

Tel: +91-7152-287700 | Reg No: MH-MED-1998-00187

PATIENT DISCHARGE SUMMARY & CLINICAL RECORD

CONFIDENTIAL — Contains identifiable patient information. For authorized clinical use only.

SECTION 1 — PATIENT IDENTIFICATION

Patient Full Name	Patient ID (MRN)
Priya Subramaniam	JNMC-WD-2024-03341
Date of Birth	Age
22/07/1995	29 years
Sex	Blood Group
Female	O-
Residential Address	
12, Shivaji Nagar, Near Railway Station, Wardha - 442001, Maharashtra, India	
Contact Number	Emergency Contact
+91-70200-44321	Ravi Subramaniam (Father) — +91-70200-99876
Insurance ID	Nationality
PMJAY-MH-2024-776123	Indian

SECTION 2 — ADMISSION DETAILS

Admission Date	Discharge Date
18/10/2024	25/10/2024
Ward / Unit	Attending Physician
Neurology Ward — Bed 7A	Dr. Anjali Mehta (DM Neurology)
Referring Physician	Department
Dr. Ramesh Borde (General Physician, Wardha)	Neurology
Reason for Admission	

Progressive bilateral lower limb weakness over 3 weeks, ascending pattern. Unable to walk independently for 2 days.

SECTION 3 — PRESENTING SYMPTOMS

- Progressive bilateral lower limb weakness — ascending pattern (3 weeks)
- Tingling and numbness in both feet, now extending to knees
- Areflexia — absent deep tendon reflexes in lower limbs
- Mild facial weakness (right side)
- Difficulty swallowing (dysphagia) — onset Day 5
- Respiratory rate trending up — monitored for respiratory involvement

Symptom Onset	Duration
27/09/2024	3 weeks progressive, acute deterioration last 2 days

SECTION 4 — DIAGNOSIS

Primary Diagnosis
Guillain-Barre Syndrome (GBS) — Classic AIDP variant
Secondary Diagnosis

Recent history of gastroenteritis (Campylobacter suspected)

ICD-10 Code

G61.0 — Guillain-Barre syndrome

SECTION 5 — LABORATORY RESULTS

Marker	Value	Unit	Reference Range	Flag
CSF Protein	148	mg/dL	15–45	HIGH
CSF Cells	2	cells/uL	0–5	NORMAL
Anti-GQ1b Ab	Positive	—	Negative	HIGH
WBC	9,800	cells/uL	4,500–11,000	NORMAL
Hemoglobin	11.2	g/dL	12.0–16.0	LOW
ESR	48	mm/hr	0–20	HIGH
CRP	32	mg/L	< 10	HIGH
Sodium	134	mEq/L	135–145	LOW
Nerve Conduction Velocity	22	m/s	45–70	LOW

SECTION 6 — MEDICATIONS ADMINISTERED

Medication	Dosage	Frequency	Response
IV Immunoglobulin (IVIG)	2 g/kg over 5 days	Once (course)	Partial response
Pregabalin	75 mg	Twice daily	Good response — pain relief
Pantoprazole	40 mg	Once daily	Good response
Enoxaparin	40 mg SC	Once daily	Prophylactic — no events
Physiotherapy (active)	Daily session	Twice daily	Progressive improvement

SECTION 7 — PROCEDURES & IMAGING

- Lumbar Puncture — CSF analysis (albumino-cytological dissociation confirmed)
- Nerve Conduction Study (NCS) — Demyelinating pattern confirmed in bilateral lower limbs
- EMG — Reduced motor unit potentials
- Pulmonary Function Tests — FVC 68% (monitored for respiratory failure)
- Nasogastric tube feeding (Days 3–5 due to dysphagia)
- Daily physiotherapy — passive and active range of motion

Imaging Metadata

MRI Spine (19/10/2024): Enhancement of cauda equina nerve roots — consistent with GBS. Chest X-ray (18/10/2024): Clear lung fields. No consolidation. MRI Brain (20/10/2024): No intracranial pathology. Facial nerve enhancement noted right side.

SECTION 9 — CLINICAL OUTCOME

Discharge Outcome	Follow-up Required
Improved — Partial motor recovery	Yes — Neurology OPD in 1 week. Inpatient physiotherapy rehab for 2 weeks.
Discharge Instructions	

Continue physiotherapy daily. Avoid respiratory infections. Return immediately if breathing difficulty worsens. Pregabalin to continue for 4 weeks. Outpatient NCS repeat in 6 weeks.

SECTION 10 — CLINICAL NOTES

Physician Clinical Notes

29-year-old female presented with 3-week history of ascending weakness consistent with GBS. Preceded by gastroenteritis episode 4 weeks prior — Campylobacter jejuni suspected trigger. CSF showed classic albumino-cytological dissociation. NCS confirmed AIDP pattern. IVIG course (2g/kg over 5 days) initiated on Day 2. Respiratory function monitored closely — FVC never dropped below 60%, ICU escalation not required. Dysphagia resolved by Day 5. Lower limb strength improving — patient ambulating with support by Day 7. Discharged with outpatient physio plan. Prognosis good for full recovery within 6 months.